



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : روجيه عطيه عبدالواحد القطاوي

المحافظة : حلوان الحي / المدينة : حلوان

رقم البطاقة : 26010081601023 رقم التليفون : 01141468765

العمر (تاريخ الميلاد) : 08-10-1960 المهنة : موظف

العنوان : حلوان

تاريخ تسجيل الإستمارة : 02-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي التوقيع: (-----) المقر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني	الوظيفة: -----	التوقيع: -----
الاسم: د/ندى حسن	الوظيفة: -----	التوقيع: -----
الاسم: د/نرمين رمضان	الوظيفة: -----	التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /أ.د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : ----- قسم : -----
جهة العلاج : ----- نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: روجيه عطيه عبدالواحد القطاوي National ID: 26010081601023	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

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٢٠٢٨/٠٨/٠١

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٢٦٠١٠٠٨١٦٠١٠٢٣

٢٠٢٨/٠٨/٠١



COLOURED ECHO-CARDIOGRAPHIC REPORT

Name: روحية عطية

DATE: 2025-02-10

LVEDd	4.5	4.2 – 5.8 cm	FS	33	27 – 34 %
LVESd	2.8	2.5 – 4.0 cm	EF	68	52-72%
IVSd	1.0	0.6 – 1.0 cm	AO.	2.7	2.5 – 3.7 cm
LVPWd	1.0	0.6 – 1.0 cm*	LAD	4.0	2.0 – 4.0 cm

Comment:

- **Left ventricle:**
 - Normal internal dimensions with Normal LV systolic function, EF=68% by M-mode, same range by eyeballing.
 - RWMA: No at rest
 - Grade I Diastolic dysfunction.
- **Left atrium and aortic root:**
 - Dilated LA diameter.
 - Normal aortic root diameter.
- **Right side of the heart:**
 - Normal dimensions with normal RV systolic function.
- **Cardiac valves:**
 - **Mitral valve:** Normal motion, sclerotic posterior annulus with Mild MR
 - **Aortic valve:** Mildly sclerotic with normal flow
 - **Tricuspid valve:** normal structure with normal flow
 - **Pulmonary valve:** Normal structure and flow
- **Pericardium:** No pericardial effusion.
- **Intracavitary masses or thrombi:** No intracavitary mass or thrombi
- **Cardiac septa:** Normal interventricular and interatrial septum.

Impression

Within Normal Echo-Doppler study, grade I DD

Dr. mohamed hamdy MSc

Dr. Mohamed Magdy, resident

Name:	روحيه عطيه	
MR. :	3942	
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient presents with a clonal B-lymphoid population identified on peripheral blood flow cytometry (IPT ON PB 43 % CLONAL B LYMPHOID CELLS). Imaging and surgical pathology of a right axillary lymph node excision revealed CLL/SLL. The patient is 65 years old, has hypertension, and is hepatitis C virus (HCV) positive. The last HCV PCR performed in October 2025 was negative. The patient had a maculopapular, intensely itchy skin eruption in 2020, which was attributed to HCV and managed by dermatology. The patient has a history of del 17B negative disease and IGVH status was not performed. The patient underwent bendamustine therapy followed by six cycles of CVP (cyclophosphamide, vincristine, prednisone). The last CVP cycle was administered six months prior to the current visit. The patient now reports pruritus as a new complaint. The patient is now diagnosed with progressive SLL with pruritus, B symptoms, and pancytopenia. Physical examination reveals a rash over the upper thighs and hepatomegaly. The current treatment plan is initiation of ibrutinib through Mersal.		
تحريرا في ,		
	الوقت :	التاريخ : 19-09-2026
	رضوى محمد محمود بكري	الطبيب المعالج :
يعتمد ,		



Bone Densitometry Report: Thursday, February 13, 2025

Referring Physician: Dr Ahmed Saad

PATIENT:

Name: RAWHYA ATIYA ABDL WAHED,
Patient ID: (not specified) **Birth Date:** 08/10/1960 **Height:** 154.0 cm
Sex: Female **Measured:** 13/02/2025 **Weight:** 65.0 kg
Indications: **Fractures:** **Treatments:**

ASSESSMENT:

With a Z-score of 1.1, this patient's BMD is slightly higher than the normal limits for their age and sex.
With a Z-score of 0.5, this patient's BMD is considered within normal limits relative to their age. Even so, they may be considered osteopenic or osteoporotic, which is normal for this age.
With a Z-score of 0.1, this patient's BMD is considered within normal limits relative to their age. Even so, they may be considered osteopenic or osteoporotic, which is normal for this age.

Site	Region	Measured Date	Measured Age	Age-Matched Z-score	BMD
AP Spine	L1-L4	13/02/2025	64.3	1.1	1.097 g/cm ²
Left Femur	Total	13/02/2025	64.3	0.5	0.935 g/cm ²
Left Forearm	Radius 33%	13/02/2025	64.3	0.1	0.773 g/cm ²

RECOMMENDATION:

PLEASE ENTER RECOMMENDATION TEXT.

FOLLOW-UP:

PLEASE ENTER FOLLOW-UP TEXT.

Based on these results, a follow-up exam is recommended in February 2027

(not specified)

Bone Densitometry Report: Thursday, February 13, 2025

Referring Physician: Dr Ahmed Saad

PATIENT:

Name: RAWHYA ATIYA ABDL WAHED,
Patient ID: (not specified) **Birth Date:** 08/10/1960 **Height:** 154.0 cm
Sex: Female **Measured:** 13/02/2025 **Weight:** 65.0 kg
Indications: **Fractures:** **Treatments:**

ASSESSMENT:

The BMD measured at AP Spine L1-L4 is 1.097 g/cm² with a T-score of -0.8. Bone density is up to 10% below young normal. This patient is considered normal according to World Health Organization (WHO) criteria.
 The BMD measured at Femur Total is 0.935 g/cm² with a T-score of -0.5. Bone density is up to 10% below young normal. This patient is considered normal according to World Health Organization (WHO) criteria.
 The BMD measured at Forearm Radius 33% is 0.773 g/cm² with a T-score of -1.2. This patient is considered to have low bone mass according to World Health Organization (WHO) criteria. Bone density is between 10 and 25% below young normal. Treatment is advised.

Site	Region	Measured Date	Measured Age	WHO Classification	Young-Adult T-score	BMD
AP Spine	L1-L4	13/02/2025	64.3	Normal	-0.8	1.097 g/cm ²
Left Femur	Total	13/02/2025	64.3	Normal	-0.5	0.935 g/cm ²
Left Forearm	Radius 33%	13/02/2025	64.3	Osteopenia	-1.2	0.773 g/cm ²

World Health Organization (WHO) criteria for post-menopausal, Caucasian Women:
 Normal T-score at or above -1 SD
 Osteopenia T-score between -1 and -2.5 SD
 Osteoporosis T-score at or below -2.5 SD

FOLLOW-UP:

Patient follow up in February 2027

(not specified)

التاريخ: 2025/2/5

الاسم: روحية عطية

***LIVER:** average size , regular surface, **coarse echopattern**, homogeneous texture, normal hepatic veins, and portal vein is not dilated measuring 11 mm, no intrahepatic biliary radical dilation (IHBRD), with no hepatic focal lesions.

***GALL BLADDER:** average in size, normal wall thickness, there is no gravels or sludge are seen inside, C B D is not dilated.

***SPLEEN:** bulky in size measuring 14cm, with homogenous echo pattern and splenic vein is not dilated measures 5mm.

***RT. KIDNEY:** average size, normal parenchymal thickness, **increased echogenicity**, with good parenchymal pelvis differentiation. No calculi or back pressure changes.

***LT. KIDNEY:** average size, normal parenchymal thickness, **increased echogenicity**, with good parenchymal pelvis differentiation. No calculi, no back pressure changes.

***NO** ascites, no masses, no lymphadenopathy.

***PANCREATIC AND PARA AORTIC AREA:** masked by gases

CONCLUSION:

Coarse liver .

Bulky spleen .

Bilateral grade 1 nephropathy .

Signature
AHMED SAAD

Dr. Ahmed El-Habashi , MD , PhD , FIAC
Professor of Pathology;
NCI , Cairo University
International Board of Cytopathology
Fellow of International Academy of Cytology
Fellow Tulane Univ. Hospital, USA
Fellow Groningen Univ. Hospital, Holland

أ.د. أحمد حسين الحبشى
أستاذ الباثولوجى - المعهد القومى للأورام - جامعة القاهرة
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زميل جامعة خرونينجن- هولندا

Patient Name: روجيه عطيه عبدالواحد

Age: 64 Y-O

Gender: Female

Clinical History: Generalized lymphadenopathy

Nature of Specimen: Right axillary lymph node excision biopsy for Immunohistochemistry

Referring Prof. Doctor: M. Gad

Date of Specimen Receiving: 20/2/2025

Date of Reporting: 23/2/2025

Immunohistochemistry Report

Gross Description:

Sections were immunostained for Ki-67, CD23, B- Marker (CD20), CD5, Cyclin-D1 and Pan-T marker (CD3), using ABC technique with antigen retrieval and DAB chromogen.

Microscopic Description:

The small lymphoid cells showed positive reactivity for CD20, CD23 and CD5 and negative for Cyclin-D1 and Pan-T marker (CD3)

Ki-67 labeled about 10% of cells.

Diagnosis

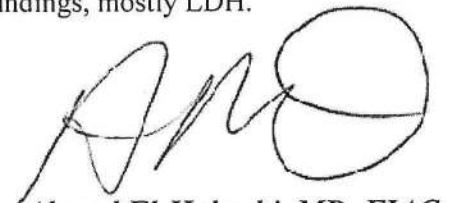
Lymph Node, Sonar Guided CNB, Immunohistochemistry:

**Immunophenotypic pattern compatible with
Small B-cell lymphoma, "low grade" (CLL/WLL).**

Comment:

Immunostainings results should be considered in the context of total amount of relevant data including clinical, radiological and tumor biochemical marker serum level findings, mostly LDH.

Ps. 6 slides enclosed.


Ahmed El-Habashi, MD, FIAC

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تليفون: ٢٣٩٢٩٩٣٧ موبایل: ٠١٦٦٧٣٦٦٠٨
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عضو الجمعية الأمريكية للسيتولوجى
زميل جامعة تولين- نيو اورليانز- امريكا
زميل جامعة خرونينجن- هولندا

Patient Name: روجيه عطيه عبدالواحد

Age: Y-O

Gender: Female

Clinical History: Generalized lymphadenopathy.

Nature of Specimen: Right axillary lymph node Excision biopsy

Referring Prof. Doctor: M. Gad

Date of Specimen Receiving: 15/2/2025

Date of Reporting: 19/2/2025

Surgical Pathology/Cytopathology Report

Gross Description

Specimen received in formalin and labeled "Right axillary lymph node, excision biopsy" It consisted of one defined nodules 2.5x2 cm. Cut section was soft tan and pinkish.
Sections in 2 cassettes.

Microscopic Description:

Sections showed lymph nodes parenchyma displaying effaced nodal architecture with effaced subcapsular sinuses and involvement by diffuse monotonous predominantly small sized round lymphoid looking monotonous mononuclear cells existed in diffuse pattern and displaying hyperchromasia, and conspicuous cytoplasm as well as possible vague proliferating centers. Focal fat infiltration was seen.

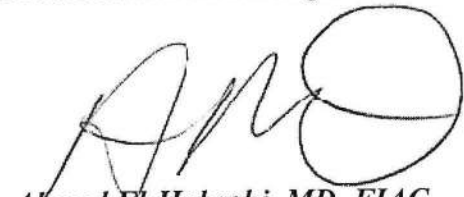
Diagnosis:

Right Axillary Lymph Nodes, Excision Biopsy:

Diffuse small lymphoid looking cell proliferation, likely lymphomatous.
For immunophenotyping confirmation.

Comment:

Please correlate with clinical, radiological, hematological and LDH serum level findings.


Ahmed El-Habashi, MD, FIAC

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برج ١- برعى بلازا- المحور الرئيسى- بجوار مسجد الحصرى مباشرة- ٦ أكتوبر
تليفون : ٢٣٩٢٩٩٣٧ موبايل : ٠١٦٦٧٣٦٦٠٨
E-Mail: elhabashiahmed@yahoo.com

Patient Name:	RAWHEIA ATTYA	Age	
Ref. Phys:			06/02/2025

**AXIAL CT SCAN OF THE NECK, CHEST, ABDOMEN AND PELVIS WAS
PERFORMED AFTER IV CONTRAST ENHANCEMENT AND GUT
OPACIFICATION**

Neck

- • Bilateral multiple enlarged cervical lymph nodes.
- The visualized oral cavity including the tongue base and oral cavity floor are within normal limits with no obvious mass lesion.
- The nasopharyngeal , oropharyngeal and laryngopharyngeal air spaces are preserved. The laryngeal skeleton look normal .
- The para pharyngeal space including the carotid sheathes are within normal limits .
- The salivary gland including the parotid and submandibular glands are grossly unremarkable .
- The para spinal musculature and retro pharyngeal spaces are within normal.

Chest

- • Bilateral multiple axillary lymph nodes . The largest 2.7cm of right side.
- Apprant right sub clavial artery .
- The lung fields are clear with no obvious mass lesion , interstitial or alveolar changes .
- The pleural sacs and chest wall are grossly unremarkable .
- The visualized mediastinal and hilar vascular structures are within normal limits with no obvious mass lesion.

Abdomen & Pelvis

- • Hepatosplenomegaly .
- • Multiple small lymph nodes at celiac, peri pancreatic and left para aortic groups .
- The retroperitoneal structures including the kidneys and adrenal glands are within normal limits with no obvious mass lesion or significant lymph node enlargement . The opacified bowel loops including the stomach are regularly filled with no obvious mass lesion or significant mural thickening.
- No abnormality could be detected within the pelvic organs .
- No bony or musculature abnormality has been shown .

IMPRESSION.

- Bilateral enlarged cervical lymph nodes.
- Bilateral axillary lymph nodes.
- Hepatosplenomegaly and multiple abdominal small lymph nodes for clinical , lab correlation and further evaluation to exclude lymphoma.

Best Regards,

Dr. Mahmoud El-Shamy (M.D)



Date : 20/05/2025	Patient Name: Rawheya Atteya Abd Al Wahed
Age : 64 Year(s)	Exam : ultrasound ABDOMEN& PELVIS
Pat. ID: 99057	Medical ID: 610/2025

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:**

Liver: -

- Enlarged in size (about 18 cm at the right MCL), smooth outline, bright coarse echopattern. No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent .

Gall Bladder: -

- Average size , normal wall thickness, no stones..
- CBD is not dilated.

Spleen: -

- Border line in size (About 13 cm), displaying homogenous echopattern.

Kidneys: -

Are of normal size and site , normal left echogenicity , increased right echogenicity no stone or backpressure changes , no solid or cystic lesion.

-Para-aortic and pancreatic area: no gross lesion

URINARY BLADDER: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.

Uterus: -: bulky corresponding to age of patient average Endometrial thickness ,heterogenous ,no focal lesions

No free fluid in the Douglas pouch.

Opinion:

Chronic parenchymatous liver disease with Bright hepatomegaly.

Border line spleen

Bulky uterus for TVUS

Not Approved